

ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #02

ADVANCED BULKING CYCLE WITH PEPTIDES

16-Week Mass Protocol + GHRP/GHRH Integration

16 Weeks · 6 Chapters · Advanced Level · Peptide Stack

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DISCLAIMER

This guide is for educational purposes only and does not constitute medical or professional advice. Always consult a qualified healthcare provider before starting any supplementation, pharmaceutical, or training protocol. All information is provided for harm-reduction and educational purposes. Individual results vary.

CHAPTER 1 — BULKING CYCLE OVERVIEW

"Mass is built in surplus, revealed in deficit. The quality of your bulk determines the quality of your cut."

This advanced 16-week bulking cycle pairs traditional anabolic compounds with growth-hormone-releasing peptides (GHRPs and GHRHs) for maximal lean mass accrual with minimal fat gain. Peptide use allows a more controlled hormonal environment, supporting IGF-1 elevation, improved sleep, and enhanced recovery alongside the traditional compounds.

Compound	Dose	Frequency	Role
Testosterone Enanthate	500–750mg/wk	Mon + Thu	Mass base
Deca-Durabolin (NPP)	400mg/wk	Mon + Thu	Joint support, mass
Dianabol (Methandrostenolone)	30–50mg/day	Daily oral (wk 1–6)	Fast-start kickstart
GHRP-6 or Ipamorelin	100–200mcg	3× daily subQ	GH pulse, hunger stimulation
CJC-1295 (no DAC)	100mcg	3× daily with GHRP	GHRH synergy
Mk-677 (Ibutamoren)	12.5–25mg/day	Oral nightly	GH secretagogue, sleep quality

■ **Peptides are injected subcutaneously in the abdomen, 20 minutes before meals and before bed. Use bacteriostatic water for reconstitution.**

CHAPTER 2 — PEPTIDE INTEGRATION PROTOCOL

GH-releasing peptides work synergistically — a GHRP creates a pulse, while a GHRH (like CJC-1295) amplifies its magnitude. The combined effect can increase GH pulse amplitude by 4–10× baseline.

Time	Peptide	Dose	Notes
6:30 AM (fasted)	GHRP-6 + CJC-1295	100mcg each	Pre-breakfast, potentiates GH
Post-training	Ipamorelin + CJC-1295	150mcg each	Anabolic window amplification
11:00 PM (pre-sleep)	Ipamorelin + CJC-1295	100mcg each	Sleep GH pulse — do not eat 2h before

- Store reconstituted peptides at 2–8°C; use within 28 days
- GHRP-6 causes significant hunger — time injection before a large meal intentionally
- Ipamorelin is hunger-neutral — preferred for users who struggle with appetite control
- MK-677 causes water retention and may increase fasting glucose — monitor if diabetic risk
- IGF-1 bloods at baseline and Week 8 to confirm peptide efficacy

CHAPTER 3 — WEEK-BY-WEEK TRAINING

Weeks	Split	Volume	Key Lifts	Recovery
1–4	PPL 6 days	Moderate (12–16 sets)	Squat, Bench, Deadlift	Full rest Sunday
5–8	PPL + Arms	High (16–20 sets)	Add heavy Romanian DL	Deload Sat
9–12	Upper/Lower 4 days	Very high (20+ sets)	Maximum load progressive	Active recovery 3 days
13–16	PPL + Specialisation	High with intensity	Weak point focus	Strategic deload wk 15

■ ***Progressive overload is the single non-negotiable variable. If you are not adding weight or reps each week, your nutrition is the problem — not the training.***

CHAPTER 4 — CALORIC SURPLUS & MACROS

Bulk Phase Macro Targets

Body Weight	TDEE Estimate	Surplus	Protein	Carbs	Fats
70kg	3000 kcal	+500 = 3500	210g	490g	80g
80kg	3400 kcal	+500 = 3900	240g	550g	90g
90kg	3800 kcal	+600 = 4400	270g	630g	100g
100kg	4200 kcal	+600 = 4800	300g	700g	110g

- Eat every 3 hours — 5–6 meals. Anabolic hormones demand constant substrate delivery
- Indian bulk staples: rice, roti, daal, paneer, chicken, eggs, banana, full-fat milk
- Mass gainer only if reaching calorie targets from food alone is not feasible
- Creatine monohydrate: 5g daily (non-negotiable for mass phase)
- Digestive enzymes with large meals to improve nutrient assimilation

CHAPTER 5 — RECOVERY OPTIMISATION

"Growth happens outside the gym. Recovery is not rest — it is the actual work."

Recovery Tool	Frequency	Duration	Benefit
Sleep	Every night	8–9 hours	GH pulse, protein synthesis
Foam Rolling	Post-training	10–15 min	Fascial release, blood flow
Contrast Shower	4× per week	10 min	Inflammation reduction
Massage	Weekly	60 min	Tissue quality, CNS recovery
Deload Week	Every 4 weeks	7 days 50% volume	Tendon, CNS restoration

■ **Peptide-enhanced sleep quality should produce noticeably deeper, more restorative sleep within 2–3 weeks. If not, reassess dosing timing.**

CHAPTER 6 — CYCLE END ASSESSMENT

Post-Cycle Metrics Checklist

- Body weight change — expected gain: 6–12kg total (3–5kg lean with clean bulk)
- Strength benchmarks — expect 10–20% increases on main lifts
- Body fat % — should remain within +2–3% of starting point with clean diet
- Waist measurement — if waist increased > 4cm, surplus was excessive
- Blood panel: testosterone (total + free), estradiol, LH, FSH, CBC, lipids, liver enzymes
- Blood pressure: target < 130/80 — if elevated, address before PCT

PCT Initiation Timeline

Compound Last Injection	Wait Before PCT	PCT Protocol
Test Enanthate (250mg)	14–18 days	Clomid 50/50/25/25 + Nolvadex 40/40/20/20
Deca (NPP)	10–14 days	Extended PCT — 6 weeks minimum
All peptides	Discontinue cycle end	Continue MK-677 through PCT (optional)